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HAVEN PAEDIATRIC CENTRE

# Working Capital and Receivables Recovery Plan

Unlocking the cash already earned


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**PREPARED FOR**
The board, Haven Paediatric Centre

**PREPARED BY**
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**WORKSTREAM**
1 of 4, Diagnostic Audit

**DATE**
6 October 2026


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# A letter to the board

Dear Kabir, Mrs Alli, Dr Saliu, Dr Odedina and Mr Odum,

This is the second of the two deliverables from the diagnostic audit, and it is late. It was due alongside the audit report in August. I would rather tell you why than let it pass.

The reason is in the audit itself. We asked for the accounts receivable ageing by payer, and for claims submitted, paid, rejected and outstanding by payer. Both were marked critical in our information request. Neither was supplied, and we said so in the report: receivables ageing, denial management and collection performance were not available for review and remain the open question. A recovery plan built on figures we had not seen would have been a plan in the shape of a plan, and worth nothing to you.

So this document does two things instead. It sets out everything the recovery can proceed on today, which is more than you might expect, and it names precisely what we still need and by when. The first fortnight of work in section 6 does not depend on the missing data at all. It can start the day you read this.

One thing I want to put in front of you plainly. The money we are discussing is money Haven has already earned, for care it has already given, with staff it has already paid. It is not new revenue and it does not require a single additional patient. It is the cheapest money in the business and it has been sitting outside the hospital while the hospital worried about cash.

Dr Debo Odulana

Founding Partner, Consult for Africa

## 01 What we established

Four findings from the audit bear directly on the cash position. None of them required the missing schedules.

| PAYER CONCENTRATION | IS NOT PAYERS AT ALL | REJECTION REASONS RECORDED |
|---|---|---|
| **72.1%** | **N399,253** | **0** |
| two payers carry most of the insured revenue | two lines in the ageing schedule | no field for it exists anywhere |

**The concentration is the risk nobody has priced.** Two payers account for 72.1 per cent of insured revenue and 71.3 per cent of insured encounters. Losing either would remove roughly a quarter of Haven's total revenue. That is a commercial exposure in its own right, and it also shapes the recovery: almost all of the money is behind two relationships, so this is a job of two conversations done properly rather than a collections campaign.

**Part of the ageing schedule is not what it appears to be.** Two lines totalling N399,253 are not payers. One is a patient name. The other last appears as a claim in November 2025 and was never paid. Until those come out, every figure anybody quotes about Haven's receivables is wrong by at least that much, and probably more.

**Nothing records why a claim was refused.** There is no rejection reason field anywhere in the claims record. That is the single most consequential gap in this whole area, because it means no one at Haven can say whether claims fail for authorisation, documentation, tariff or timing. Without that, recovery is guesswork and prevention is impossible.

**The revenue record itself is a nursing handover document**, completed from memory. That is covered in the audit, and it matters here because it is the upstream source of what gets billed at all.

## 02 What we still need

These were marked critical in the original information request and have not been supplied. The recovery cannot be sized without them, though as section 6 shows, it can begin.

| What | What it answers | Needed by |
|---|---|---|
| **Accounts receivable ageing by payer** | The actual outstanding position, by age and by payer | Within 7 days |
| **Claims submitted, paid, rejected and outstanding, by payer** | The recovery rate, and where claims die | Within 7 days |
| **Cash collection and banking reports** | Whether what is collected reaches the account | Within 7 days |
| **Management accounts, last twelve months** | Service line contribution and the true margin | Within 14 days |
| **Pharmacy stock valuation and movement** | The other half of the working capital | Within 14 days |

If a schedule does not exist in a usable form, say so and we will build it with your team rather than wait for it. A partial or messy record is a finding, not a failure, and we would far rather have what exists than a tidy version assembled to look right.

## 03 The recovery, in four moves

### Move one. Clean the schedule before anyone calls anybody

Take the non-payer lines out. Agree one number with your finance lead, and have that number be the only one anyone uses. Then age it properly: current, thirty days, sixty, ninety, and beyond. Beyond ninety is where claims go to die quietly, and it is usually larger than anybody expects.

This takes a day and it is the precondition for everything after it. Walking into a payer conversation with a number you cannot defend hands them the meeting.

## Move two. Reconcile to the line, payer by payer

For each of the two large payers, match what Haven billed against what they say they owe, claim by claim. Not in total. In total is where both sides agree to disagree and nothing moves.

You will find three buckets. Claims they have paid and Haven has not recorded. Claims Haven submitted that they never received. And claims genuinely in dispute, which is the only bucket that needs a conversation.

## Move three. Make the call, properly

Ring the person, not the organisation. Go in having reconciled, because the sentence that moves money is "I have reconciled our account to the line," and almost no provider calling an insurer has.

> *I have reconciled our account to the line. We show N__ outstanding across __ claims from __ to __. Your statement shows N__. The difference is __ claims and I have the documentation for each of them ready to send today. Can we agree the undisputed portion now, and set a date for the rest?*

Agree the undisputed amount in the room and get a date for it. Settle the disputed claims one by one against evidence. Escalate on a schedule rather than on frustration: the claims team, then the medical team, then the relationship owner, with a week between each.

Given the concentration, these two relationships are worth protecting as well as collecting from. Nothing in this should read as a threat, because losing either payer costs Haven far more than the receivable.

## Move four. Stop it rebuilding

Recovery without discipline buys one month of cash and then the problem returns, usually larger. Four things hold it.

- A rejection reason recorded against every refused claim, from this month. This is the highest value change in the document and it is a field, not a system.
- A submission deadline measured in days from the encounter, tracked weekly, because a claim refused on timing has nothing to do with the care.
- Authorisation confirmed before the encounter, with a named person who chases codes.
- A monthly reconciliation per payer, by a named owner, on the same day each month. The dullness is the point.

## 04 The other half of the working capital

Receivables are the visible half. Stock is the half sitting on a shelf.

Haven's own cash is tied up in pharmacy inventory it has already bought and not yet dispensed. Moving the pharmacy to a consignment arrangement, where the supplier owns the stock in Haven's pharmacy and is paid on dispense, releases that cash and moves the expiry risk to the party better placed to carry it.

Two conditions on that. Consignment with no reorder points still produces stockouts, and consignment without counting still produces shrinkage. The supplier would be carrying your capital, not your management. And the audit found no rostered pharmacist on dispensing days, which has to be resolved first: a consignment arrangement makes the dispensing record commercially significant as well as clinically required.

The stock valuation and movement report is one of the outstanding items in section 2. Until it arrives this half cannot be sized either.

## 05 What this is worth

We are not going to put a recovered figure in this document, because we have not seen the ageing schedule and a number invented here would be quoted back to us for a year.

What we can say with confidence is the shape. The receivable sits behind two relationships rather than twenty, which makes it tractable. A meaningful share of what is outstanding is usually recoverable through reconciliation alone, before any dispute is argued, because both sides are working from records that have drifted apart. And the prevention work costs nothing beyond attention.

We will size it precisely within a week of the schedules arriving, and we will put the number in front of you with its workings rather than as an assertion.

## 06 The first fortnight, starting now

None of this waits for the missing data.

| When | What | Who |
|---|---|---|
| **Days 1 to 2** | Strip the non-payer lines. Agree one receivables number. Age it properly | Head of Finance |
| **Days 1 to 3** | Add a rejection reason field and begin recording it on every refused claim | Admin and Billing Officer |
| **Days 2 to 5** | Pull the two large payers' statements and begin the line by line reconciliation | Head of Finance, with us |
| **Day 5** | Issue the outstanding schedules in section 2, or tell us they do not exist | Head of Admin and Operations |
| **Days 6 to 10** | The first payer conversation, reconciled, with a date agreed for the undisputed amount | Us, with your Head of Finance |
| **Days 8 to 12** | The second payer conversation on the same basis | Us, with your Head of Finance |
| **Day 12** | Roster a pharmacist on every dispensing day and begin the stock count | Head of Admin and Operations |
| **Day 14** | Report back to the board: recovered, agreed, disputed, and the sized position | Us |

## 07 What we need from you

Three things, and only the first is urgent.

Nominate one person on your side who owns this. On the evidence of the roster that is your Head of Finance, working with the Admin and Billing Officer. Recovery fails when it is everybody's job.

Release the schedules in section 2, or confirm within five days that they do not exist in usable form, so we stop waiting and start building them with your team.

Decide, as a board, that the rejection reason field goes in this month. It is the smallest change in this document and it is the one that stops this happening again.

I am a call away on any of it.

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